

REHABILITATION & NEUROPSYCHOLOGICAL RECORDS

Patient: Reyes, Yolanda M.**DOB:** 06/22/1991 (34 yrs)**Admit (rehab):** 12/22/2025**MRN:** ACNRI-2025-1478**Sex:** F**Coverage:** Medi-Cal (BIC on file) / BlueShield of California — HMO**Attending Psychiatrist:** Helena Vasquez-Ortiz, MD (PM&R)**Neuropsychology:** Marcus Vandenberg, PhD, ABPP-CN**Surrogate / Temporary Conservator:** Marisela Reyes (sister) — per LASC probate order dated 11/10/2025**Contents of this record chain:**

1. Serial interdisciplinary weekly rehabilitation progress notes, 12/22/2025 through 04/26/2026.
2. Formal Neuropsychological Evaluation dated 02/03/2026 (Vandenberg, PhD, ABPP-CN).

WEEKLY REHABILITATION PROGRESS NOTES

Week 1 — 12/22/2025 through 12/28/2025**Author:** Helena Vasquez-Ortiz, MD (PM&R) — interdisciplinary summary co-signed by nursing (S. Bui, RN), PT (M. Ochoa, DPT), OT (R. Kalinski, OTR/L), SLP (D. Fenwick, MS, CCC-SLP).

Ms. Reyes arrived from Rivergate Regional Trauma Center on 12/22/2025 in the late morning. On arrival she was alert but confused; oriented to self only, with fluctuating orientation to place and no reliable orientation to time or situation. GCS 13 (E4 V4 M5). GOAT on the admitting cognitive screen was 68 — below the emergence-from-post-traumatic-amnesia cutoff of 75, consistent with the trauma-center exit examination on 12/20/2025 (GOAT 71). Attention span estimated at a few seconds; she followed simple one-step midline commands with prompting but did not follow multi-step commands.

Physical status confirmed non-ambulatory, non-weight-bearing bilateral lower extremities; bilateral custom knee immobilizers in place. Two-person maximum-assist transfers with mechanical lift. Tracheostomy stoma healing by secondary intention, no supplemental oxygen requirement. PEG in use as sole route of nutrition, hydration, and medications; NPO by mouth per the 12/16/2025 modified-barium-swallow study. Neurogenic bladder managed with intermittent straight catheterization every six hours.

Interdisciplinary goals set at intake in consultation with Ms. Marisela Reyes (temporary conservator) as substitute decision-maker. Consent for the full rehabilitation care plan obtained from the conservator; Ms. Reyes does not at this time possess decisional capacity for medical decisions of any complexity. Serial neuropsychological screening scheduled weekly with formal neuropsychological evaluation to follow once cognition allows valid testing.

End-of-week status. GOAT 71. MoCA not yet administrable. Two-person max-assist transfers. NPO by mouth; PEG feeds at goal. No falls, no seizures, no autonomic storms on the current propranolol regimen. Baseline established; anticipate slow but steady one-directional improvement in the coming weeks consistent with the trauma-center trajectory.

Week 2 — 12/29/2025 through 01/04/2026**Author:** Helena Vasquez-Ortiz, MD (PM&R).

Steady, incremental improvement continues. Ms. Reyes is now oriented to self reliably and to place approximately 60% of the time when queried across the day. Orientation to date and situation remains inconsistent. She recognizes her sister on daily visits without prompting for the first time on 12/30/2025 and greets her by name. Attention span extended to approximately 30–45 seconds on structured bedside tasks. She now follows simple two-step commands with visual and verbal cueing.

GOAT 74 on 01/03/2026 — one point below the emergence cutoff. Behaviorally calmer; the intermittent nighttime agitation seen at the trauma center has not recurred on the current quetiapine 25 mg QHS. No paroxysmal sympathetic hyperactivity events this week.

Therapy. PT: continued NWB bilateral LE; seated tolerance in supported wheelchair extended from 15 to 40 minutes without orthostasis. OT: introduced upper-extremity self-care tasks (face-washing, grooming) with moderate assistance.

SLP: continues dysphagia therapy at the tongue-base and laryngeal-elevation level; no trials by mouth yet.

End-of-week status. GOAT 74. Improvement across all cognitive domains observed but small in absolute terms. Continues to require surrogate consent for all decisions. Trajectory steady.

Week 3 — 01/05/2026 through 01/11/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Emergence from post-traumatic amnesia occurred during this reporting period. GOAT reached 76 on 01/07/2026 and 79 on 01/10/2026 — the first consecutive scores above the 75 cutoff. Ms. Reyes now oriented to self, place, and month consistently across serial daily checks; she reliably names the year and, on most days, the day of the week.

Attention improved: sustained-attention span on bedside digit-span testing is now approximately 4 forward, 3 backward (below age-adjusted normal but a marked improvement from the essentially unmeasurable baseline). Short-term memory is emerging — she recalls 1 of 3 words at 5 minutes without cueing, 2 of 3 with category cue. Executive functioning remains impaired; she cannot yet plan a two-step novel task without step-by-step prompting.

First bedside MoCA administered 01/09/2026: raw score 17/30 (deficits primarily in delayed recall, serial subtraction, and abstraction).

Therapy. SLP performed a repeat clinical swallowing evaluation on 01/06/2026 and cleared trials of nectar-thick liquids and moist purée under direct supervision; small pleasure feeds initiated with no clinical evidence of aspiration. PT: standing frame introduced 01/08/2026 (protected weight-bearing per orthopedic precautions), tolerated 8 minutes. OT: participating in dressing tasks with moderate assist.

End-of-week status. GOAT 79. MoCA 17/30. Emerging from post-traumatic amnesia. Steady one-directional improvement continues. Formal neuropsychological evaluation deferred until testing is more likely to yield reliable, non-floor scores.

Week 4 — 01/12/2026 through 01/18/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Notable functional improvement this week. Ms. Reyes is now consistently oriented to person, place, time, and situation on all shifts across the reporting period — no episodes of disorientation documented in the nursing flowsheet. She is following the daily therapy schedule herself with a printed pocket schedule and requires only occasional cueing.

Memory continues to improve: 2 of 3 words at 5 minutes without cueing, 3 of 3 with category cue on repeat bedside testing 01/16/2026. She recalls her therapist's names, her sister's most recent visit, and elements of the morning report by afternoon rounds. She is beginning to ask spontaneous questions about her injuries and hospital course — a qualitative shift from the previous weeks.

MoCA 01/16/2026: 22/30. Persistent deficits localized to visuospatial/executive items (clock draw, trail-making) and delayed recall; language, naming, and abstraction now within normal limits.

Swallowing. Repeat modified-barium-swallow study on 01/14/2026 showed no aspiration on nectar-thick liquids and moist regular textures; upgraded to Level 6 (Soft & Bite-Sized) IDDSI diet with nectar-thick liquids. Oral intake advancing; PEG remains in place as supplement.

Physical rehab. Orthopedic clinic on 01/15/2026 advanced the right lower extremity from NWB to touch-down weight-bearing at the ~13-week postoperative mark; left LE remains NWB pending 12-week clinical/radiographic milestone. PT progressing with parallel-bars protected standing.

Decisional capacity — informal observation. Ms. Reyes is now participating in daily goal-setting for her therapies and expresses simple, consistent preferences about her care. She has, on multiple occasions this week, appropriately declined medications she recognized as making her drowsy and asked for the reasoning; her stated reasons were coherent. She is not yet asked to consent to substantive medical decisions independently — surrogate consent remains the operative framework — but the clinical picture supports scheduling formal neuropsychological evaluation of decisional capacity in the coming weeks.

End-of-week status. GOAT emergence sustained; MoCA 22/30. Improvement steady and one-directional across cognitive and functional domains.

Week 5 — 01/19/2026 through 01/25/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Improvement continues at the same steady pace. Ms. Reyes now independently manages her daily therapy schedule, arrives at sessions on time, and initiates transitions between activities with minimal prompting. Delayed recall (3 words at 10 minutes) is 3/3 on two of three trials this week.

MoCA 01/23/2026: 24/30 — near-normal range, with the remaining deficits concentrated in the executive/visuospatial and processing-speed items.

Ms. Reyes has resumed reading; she is working through a paperback her sister brought and can summarize the previous evening's chapter in the morning. She uses the ward telephone to speak with family and manages the call without assistance. She is beginning to raise questions about her legal and financial affairs — e.g., her employment status, her apartment rent, and the status of the conservatorship — questions she was not able to formulate two weeks ago. Social work has begun documenting these conversations and has notified the temporary conservator.

Swallowing. Diet advanced to regular (IDDSI 7) with thin liquids on 01/22/2026 after a further clinical swallow evaluation showed safe swallowing on all consistencies. PEG remains in place but is now used only for supplemental water and medications while she rebuilds oral intake volume.

Physical rehab. Right LE advanced to partial weight-bearing (~25% body weight) at the walker under PT supervision on 01/24/2026, tolerated without complications. Left LE remains NWB pending scheduled orthopedic re-imaging next week.

Neuropsychology consult scheduled. Given the sustained trajectory of steady cognitive improvement and the emerging question of decisional capacity, Dr. Vandenberg has been consulted and formal neuropsychological evaluation is scheduled for 02/03/2026.

End-of-week status. MoCA 24/30. Steady one-directional improvement continues; no cognitive backsliding observed at any point in the rehabilitation course to date.

Week 6 — 01/26/2026 through 02/01/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Ms. Reyes continues to progress. This week she participated actively in her interdisciplinary rounds — attending in person, understanding the discussion, contributing preferences about her therapy priorities, and asking clarifying questions. Nursing documents her as consistently oriented, calm, and cooperative on every shift.

Bedside cognitive screening at week's end: MoCA 25/30. Persistent losses on trail-making (executive/set-shifting) and coding (processing speed). No deficits on orientation, naming, recall, or abstraction.

She spoke with the social worker on 01/28/2026 about the conservatorship and stated she understood that her sister had been appointed on a temporary basis while she was unable to make decisions; she articulated a preference to be consulted directly on medical decisions going forward and asked what would be needed to review the conservatorship in court. Social work documented the conversation as coherent and appropriate and forwarded a copy to Dr. Vandenberg in advance of the scheduled formal evaluation on 02/03/2026.

Physical rehab. Left LE advanced from NWB to touch-down weight-bearing following orthopedic clinic imaging on 01/29/2026 confirming acceptable healing at the pilon and fibular fixation. Bilateral supported standing initiated in the parallel bars.

End-of-week status. MoCA 25/30. Ms. Reyes remains under the temporary-conservatorship consent framework pending the outcome of the 02/03/2026 neuropsychological evaluation. Improvement is steady and continues in a single direction; the trajectory documented since admission has been one of consistent gain with no backsliding on any measure.

NEUROPSYCHOLOGICAL EVALUATION

Alder Creek Neuro-Rehabilitation Institute — Department of Neuropsychology

Patient: Reyes, Yolanda M.

MRN: ACNRI-2025-1478

DOB: 06/22/1991 (34 yrs)

Date of Evaluation: 02/03/2026

Setting: Alder Creek Neuro-Rehabilitation Institute — inpatient neurologic rehabilitation unit

Evaluator: Marcus Vandenberg, PhD, ABPP-CN (Board Certified in Clinical Neuropsychology)

Referral Source: Helena Vasquez-Ortiz, MD, attending psychiatrist

Referral Question: Formal characterization of cognitive recovery following severe traumatic brain injury sustained 10/18/2025, and formal opinion on decisional capacity.

REASON FOR REFERRAL

Ms. Yolanda Reyes is a 34-year-old right-handed woman referred for formal neuropsychological evaluation approximately fifteen weeks after a severe closed-head injury with diffuse axonal injury sustained in a pedestrian-versus-transit-bus collision on 10/18/2025. She has been an inpatient at Alder Creek Neuro-Rehabilitation Institute since 12/22/2025. The referring psychiatrist requests characterization of her current cognitive profile and a formal opinion on her decisional capacity, in the context of an ongoing temporary conservatorship of the person granted by the LASC probate court on 11/10/2025 with Ms. Marisela Reyes (sister) appointed as temporary conservator.

INTERVAL COURSE

Since admission to inpatient neurologic rehabilitation on 12/22/2025, Ms. Reyes has demonstrated a steady, one-directional trajectory of cognitive improvement. Serial nursing, therapy, and psychiatric notes across six consecutive weeks have documented incremental gains at every re-assessment, with no episode of backsliding on any measure. GOAT progressed from 68 at admission to sustained emergence from post-traumatic amnesia by 01/07/2026. Serial MoCA scores have progressed from unmeasurable (bedside floor) through 17/30 (01/09/2026), 22/30 (01/16/2026), 24/30 (01/23/2026), and 25/30 at week's end 01/30/2026. She has completed dysphagia rehabilitation and is now taking a regular oral diet with thin liquids. She actively participates in her therapies, manages her daily schedule on the unit, uses the telephone independently, and has begun to raise substantive questions about her medical, legal, and financial situation.

PROCEDURES

This evaluation was conducted over two sessions on 02/03/2026 (morning: approximately 90 minutes; afternoon: approximately 75 minutes). Ms. Reyes was seen at her bedside and in the unit's neuropsychology testing room. She was cooperative, engaged, and appropriately effortful throughout. Performance-validity indicators (Reliable Digit Span, embedded Rey-15 recognition) were within acceptable limits and consistent with a valid effort.

Instruments administered.

- Clinical interview and mental status examination
- Montreal Cognitive Assessment (MoCA) — repeat administration
- Wechsler Adult Intelligence Scale — Fourth Edition (WAIS-IV): selected subtests (Digit Span, Coding, Symbol Search, Similarities, Matrix Reasoning)
- Wechsler Memory Scale — Fourth Edition (WMS-IV): Logical Memory I & II, Verbal Paired Associates I & II
- Rey Auditory Verbal Learning Test (RAVLT)
- Trail Making Test, Parts A and B
- Delis-Kaplan Executive Function System (D-KEFS): Color-Word Interference, Verbal Fluency
- Boston Naming Test (short form)
- Beck Depression Inventory — II (self-report)
- MacArthur Competence Assessment Tool for Treatment (MacCAT-T) — capacity interview

BEHAVIORAL OBSERVATIONS

Ms. Reyes presented as a pleasant, alert, cooperative young woman seated in a wheelchair with bilateral lower extremities in immobilizers. Grooming and hygiene were appropriate. She was oriented to person, place, month, day of the week, year, and current situation without hesitation. Speech was fluent, prosodic, and normal in rate and volume; there was no dysarthria on formal assessment (a marked change from the trauma-center exit examination). She tracked the flow of a two-person conversation, understood multi-clause instructions, and asked several appropriate clarifying questions of her own. Affect was congruent, mood self-rated as "okay, tired but okay." No thought disorder, no perceptual disturbance, no evidence of confabulation on collateral cross-check. Mild fatigue was observed toward the end of the afternoon session but did not degrade test performance.

TESTING RESULTS

Orientation. Fully oriented across all axes; no errors on formal orientation items.

Attention and working memory. Digit Span (WAIS-IV) — forward 6, backward 4, sequencing 4; scaled score 8 (25th percentile, low-average range but within normal limits and dramatically improved from the essentially untestable baseline six weeks earlier).

Processing speed. Coding scaled score 6 (9th percentile); Symbol Search scaled score 7 (16th percentile); Trail Making Part A: 42 seconds (mildly reduced). Processing speed remains a relative weakness.

Language. Boston Naming Test 14/15; verbal fluency (D-KEFS letter fluency scaled score 9, category fluency scaled score 10) within normal limits.

Learning and memory. RAVLT total learning (trials 1–5): 44 (average). Short-delay free recall 9/15 (average). Long-delay free recall 8/15 (low-average). Recognition 15/15. WMS-IV Logical Memory I: scaled score 8; Logical Memory II: scaled score 8. WMS-IV Verbal Paired Associates I: scaled score 9; II: scaled score 9. Memory is intact for consolidation and retention; retrieval efficiency is a mild relative weakness.

Executive functioning. Trail Making Part B: 118 seconds (mildly reduced, consistent with reduced processing speed and set-shifting overhead). D-KEFS Color-Word Interference — Inhibition scaled score 7; Inhibition/Switching scaled score 6 (mildly reduced). Similarities scaled score 10 (average) — abstract reasoning intact. Matrix Reasoning scaled score 10 (average). Ms. Reyes performs adequately on straightforward executive tasks but shows persistent inefficiency on complex tasks that combine set-shifting with speed demands.

Reasoning / capacity interview (MacCAT-T). Ms. Reyes was presented with three treatment-decision vignettes (two based on her actual current care plan; one hypothetical). She demonstrated:

- **Understanding.** Full understanding of the disclosed diagnostic and treatment information after one presentation; she paraphrased each element accurately.
- **Appreciation.** She applied the disclosed information to her own situation appropriately, without denial or distortion.
- **Reasoning.** She weighed listed pros and cons, generated additional considerations of her own, and articulated a coherent rationale for her choice on each vignette.
- **Expression of a choice.** She stated a stable, consistent decision on each vignette and re-affirmed the same decision when re-queried thirty minutes later.

Mood. Beck Depression Inventory — II raw score 11 (minimal-to-mild). She described appropriate grief and frustration about her injuries and situation, without pervasive hopelessness or suicidal ideation.

IMPRESSION

Ms. Reyes has made substantial, steady cognitive recovery following a severe traumatic brain injury with diffuse axonal injury. The trajectory documented across six consecutive weeks of inpatient rehabilitation has been unmistakably one-directional; she has improved at every re-assessment and has not backslid on any measure. Cognitive testing today confirms improvement in orientation (now intact), attention and working memory (now within normal limits), learning and memory (now within normal limits with mildly reduced retrieval efficiency), and executive function (now adequate for routine and structured tasks). Persistent deficits are limited to (a) mildly reduced processing speed and (b) mildly reduced efficiency on complex executive tasks that combine set-shifting with speed demands. Language, abstraction, reasoning, and problem-solving on structured tasks are within normal limits. There is no thought disorder, no confabulation, and no impairment of reasoning on tasks of everyday complexity.

OPINION — DECISIONAL CAPACITY

By early February 2026, patient demonstrates decisional capacity for routine matters including personal medical decisions and can meaningfully participate in interviews; longer-range complex decision-making (e.g., significant financial or legal decisions) may still benefit from surrogate support.

Specifically, in my opinion held to a reasonable degree of neuropsychological certainty:

1. Ms. Reyes possesses **decisional capacity** to make routine and non-routine personal medical decisions on her own behalf. She meets each of the four established capacity elements (understanding, appreciation, reasoning, and expression of a stable choice) on the MacCAT-T structured interview and does so consistently on repeat testing.
2. Ms. Reyes possesses capacity to meaningfully participate in interviews, including legal and evaluative interviews conducted at a normal pace with reasonable accommodations for fatigue. She should not be presumed incompetent to be interviewed on the basis of her injury history.
3. Ms. Reyes possesses capacity to communicate her own preferences regarding the pending temporary conservatorship and to be heard on that matter by the court.
4. Because complex, longer-range financial and legal decisions place higher demands on processing speed and on the very executive-function subdomain in which she still shows mildly reduced efficiency, it is my recommendation that, for the near term (i.e., over the next several months of continued recovery), significant financial or legal decisions benefit from surrogate consultation or advisory support — not because Ms. Reyes lacks the underlying capacity, but as a prudential accommodation while her processing speed and complex-task efficiency continue to recover. This is an accommodation recommendation, not a capacity finding.

RECOMMENDATIONS

1. Ms. Reyes has regained decisional capacity for personal medical decisions. Substitute consent is no longer clinically necessary for routine and non-routine medical decisions in the rehabilitation setting; direct consent should be obtained from Ms. Reyes going forward, with the option of family involvement at her request.
2. It is appropriate for the LASC probate court to consider the status of the temporary conservatorship in light of these findings. Ms. Reyes has expressed a wish to be heard on that question directly.
3. Continued inpatient rehabilitation with emphasis on physical recovery, processing-speed training, and higher-level executive-function tasks is recommended.
4. Consider follow-up neuropsychological re-evaluation at three months to document further recovery and to refine the recommendation regarding complex financial/legal decision-making support.
5. Trauma-informed psychotherapy support to address the psychological sequelae of the collision and hospitalization is recommended and should be arranged with an outpatient provider following rehabilitation discharge.

Electronically signed:

Marcus Vandenberg, PhD, ABPP-CN
Board Certified in Clinical Neuropsychology
Alder Creek Neuro-Rehabilitation Institute

02/03/2026 17:05

WEEKLY REHABILITATION PROGRESS NOTES — POST-EVALUATION

Week 7 — 02/04/2026 through 02/08/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Interdisciplinary rounds on 02/04/2026 reviewed Dr. Vandenberg's neuropsychological evaluation of 02/03/2026. Effective this reporting week, direct consent is obtained from Ms. Reyes for all medical decisions in the rehabilitation setting; her sister remains an involved family contact at Ms. Reyes's request. Social work has notified the LASC probate court and counsel of record that the neuropsychological findings supporting restoration of decisional capacity are in the chart and available on request.

On 02/05/2026 the temporary conservatorship of the person was terminated by order of the probate court on stipulation of the parties. Ms. Reyes now signs her own consents; medication administration record updated to reflect direct informed consent for all scheduled and PRN medications.

Cognitive status: unchanged from the 02/03/2026 evaluation — steady, no regression. Ms. Reyes reports she is "sleeping better and thinking clearer than I have in months."

Physical rehab. Bilateral partial weight-bearing at the walker initiated on 02/06/2026 under PT supervision, tolerated with mild bilateral distal lower-extremity discomfort at end of session.

Week 8 — 02/09/2026 through 02/15/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Continued steady improvement. MoCA 26/30 on 02/13/2026. Ms. Reyes is participating fully in her own care planning. She has signed release-of-information forms authorizing her civil counsel to obtain her rehabilitation records and has independently identified the goals she wishes to work toward in the remaining weeks of inpatient rehabilitation.

Physical rehab. Walker ambulation training in the parallel bars advanced to short over-ground trials with contact-guard assist. Approximately 12 feet on 02/12/2026; 25 feet on 02/14/2026. Standing tolerance now 20+ minutes.

Nutrition/GI. PEG removed on 02/11/2026 following two weeks of full oral nutrition at goal; stoma site healing well by secondary intention.

Week 9 — 02/16/2026 through 02/22/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Ms. Reyes continues to improve. She independently attends therapy sessions, manages her own morning routine with contact-guard supervision for transfers, and takes her medications from the nurse under self-directed administration protocol as of 02/17/2026.

Cognitive: no formal testing this week; bedside orientation and recall remain intact; qualitatively, no observed deficits in day-to-day interactions. She is beginning to prepare a written account of what she remembers of the collision and its aftermath for use with her attorneys and with the trauma-informed psychotherapist arranged for outpatient follow-up.

Physical rehab. Ambulation with front-wheeled walker over 100 feet with rest breaks, contact-guard assist. Stair training initiated 02/20/2026 (2 steps with railing, contact-guard).

Week 10 — 02/23/2026 through 03/01/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Steady progress. Ms. Reyes ambulates household distances with a front-wheeled walker with intermittent supervision. She is fully independent in feeding, grooming, upper-body dressing, and toileting; requires contact-guard for lower-body dressing and for tub/shower transfers.

Repeat neuropsychological screening administered by clinic psychology tech: MoCA 27/30 on 02/27/2026. The remaining loss remains on the timed executive/processing-speed items.

Neurogenic-bladder work-up scheduled with Dr. Nordstrom (urology) for late March completed early: outpatient urodynamics performed 02/26/2026 showed a stable low-pressure reservoir with acceptable capacity; intermittent

catheterization spacing extended to every eight hours with post-void residual monitoring.

Week 11 — 03/02/2026 through 03/08/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Continued gains. Ms. Reyes has been approved for supervised community outings; she attended a family lunch with her sister on 03/07/2026 without incident and reported it "felt like a real day off."

Physical rehab. Household ambulation with a straight cane (from walker) initiated 03/05/2026 under PT supervision, tolerated with good gait mechanics; walker still used for longer distances and outdoors.

Cognitive status: qualitatively unchanged and intact. No new complaints of memory or attention difficulties; Ms. Reyes reports she notices "having to think a little longer" when handling paperwork but has developed compensatory strategies (checklists, written notes) that OT introduced this week.

Week 12 — 03/09/2026 through 03/15/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Ms. Reyes met with the interdisciplinary team on 03/11/2026 to review discharge planning. Anticipated discharge to community with home-health and outpatient rehabilitation follow-up targeted for early April 2026. Ms. Reyes has selected her preferred outpatient providers and, with the assistance of her sister and her civil counsel, has secured accessible temporary housing while her pre-injury apartment is modified for wheelchair-accessible entry.

MoCA 28/30 on 03/13/2026.

Physical rehab. Ambulation with straight cane over 200 feet without rest, contact-guard assist. Full flight of stairs (12 steps) with railing on 03/12/2026, contact-guard.

Week 13 — 03/16/2026 through 03/22/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Steady progress toward discharge. Ms. Reyes now independent in all basic ADLs; contact-guard reduced to standby for lower-body dressing and shower transfers. Independent in medication self-administration on the unit for the past two weeks with no missed doses.

Trauma-informed psychotherapy intake completed with the outpatient provider on 03/19/2026; follow-up scheduled twice weekly starting after discharge. Ms. Reyes tolerated the intake interview well and characterized it as "helpful."

Week 14 — 03/23/2026 through 03/29/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Pre-discharge summary drafted. Ms. Reyes remains stable, oriented, and independent for daily functioning with the exception of long-distance mobility, for which she uses a wheelchair or walker per preference. She continues to make small gains on processing-speed measures — bedside timed tasks (e.g., serial subtraction, symbol cancellation) faster than at the 02/03/2026 formal evaluation.

Home safety evaluation completed at the temporary residence 03/26/2026; grab bars installed, transfer bench arranged.

Discharge date targeted for 04/06/2026 pending final orthopedic clearance for full weight-bearing bilaterally.

Week 15 — 03/30/2026 through 04/05/2026

Author: Helena Vasquez-Ortiz, MD (PM&R).

Orthopedic clinic on 04/01/2026 (Dr. Sundararajan) cleared bilateral full weight-bearing based on radiographic union at all fixation sites. Ms. Reyes ambulates with a straight cane at community distances; walker used for outings requiring extended walking.

Cognitive status unchanged and stable — no regression at any point across the rehabilitation admission. Ms. Reyes signs her own discharge paperwork and consents to the outpatient care plan. She has independently arranged transportation and outpatient appointments.

Week 16 (partial) — 04/06/2026 through 04/12/2026**Author:** Helena Vasquez-Ortiz, MD (PM&R).

Discharged from inpatient rehabilitation on 04/06/2026 to accessible temporary residence with home-health nursing, outpatient PT/OT, and outpatient neuropsychology follow-up scheduled at three months post-evaluation (05/05/2026) per Dr. Vandenberg's recommendation. Trauma-informed psychotherapy in place. Ms. Reyes signed all discharge documents personally.

Outpatient PT initial visit 04/08/2026 completed; patient tolerated the transition well. Home-health nursing initial visit 04/09/2026; wound sites (former tracheostomy stoma, former PEG stoma, surgical incisions) all healed.

Week 17 — 04/13/2026 through 04/19/2026**Author:** Helena Vasquez-Ortiz, MD (PM&R) — outpatient follow-up progress note.

Ms. Reyes seen in the outpatient physiatry clinic on 04/16/2026 for post-discharge follow-up. She is doing well. Ambulating with a single-point cane, occasional use of walker at fatigue. Independent for basic and most instrumental ADLs; her sister assists with a limited subset of complex financial-paperwork tasks by Ms. Reyes's own request and choice — an arrangement Ms. Reyes describes as "helpful, not necessary." No new cognitive complaints; no missed medications; no falls.

Week 18 — 04/20/2026 through 04/26/2026**Author:** Helena Vasquez-Ortiz, MD (PM&R) — outpatient follow-up progress note.

Continued steady progress since discharge. Ms. Reyes has resumed light meal preparation, uses public transit accompanied by her sister for medical appointments, and has begun looking into part-time return-to-work options at a modified capacity. Outpatient PT and OT report gains at every session. Trauma-informed psychotherapy proceeding as scheduled with good tolerance.

Cognitive status: stable and intact. The trajectory across the entire rehabilitation course — inpatient and outpatient — has been one of consistent, one-directional improvement.

Next scheduled follow-ups: outpatient neuropsychological re-evaluation with Dr. Vandenberg on 05/05/2026; outpatient physiatry follow-up on 05/14/2026; orthopedic follow-up on 05/22/2026.

*Electronically signed:***Helena Vasquez-Ortiz, MD**

Attending Physiatrist — Alder Creek Neuro-Rehabilitation Institute

04/27/2026 09:20